

Promotion in Place

A Roadmap for Competency-Based Time-Variable General Surgical Training to Promote Autonomy and Readiness for Independent Practice

A Work Product of the Blue Ribbon Committee II

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Disclosures

R. Hirschl: No disclosures

M.E. Goldhamer: PI, “Promotion in Place” AMA grant, MGB

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Some icons are made with ChatGPT with input of our original content

Thank you:

- Rachel Newman EdD MS, Assistant Director, Surgical Training and Development, ACS
- Peter Schmitt MA, Senior Program Administrator, ACS
- ACS: PIP Website and Pilot Program Development Support

- Confidential data slides have been removed and will be presented during the session.

PIP in General Surgery

Endorsed by and developed in collaboration with:

- Accreditation Council for Graduate Medical Education
- American Board of Surgery
- American College of Surgeons
- American Surgical Association
- Association of Program Directors in Surgery
- American Medical Association (Reimagining Residency grant)

Learning Objectives

- Provide an overview of the Promotion in Place CBTV-GME pilot and adaptations to general surgical training
- Review the explicit criteria that determine trainee qualifications for early graduation and a period of PIP sheltered independent (PIP-SI) practice
- Explore key considerations for planning schedules and practice activities for the period of PIP-SI

Agenda

- What's the Problem We Are Trying to Solve?
 - Rationale of CBME
 - Promotion in Place
- Turning Theory Into Reality Across Specialties
 - The History of CB-TV Models in the U.S. and PIP in GME
- Why General Surgery and Why Now?
- The General Surgery CB-TV PIP Model

What is the Problem We Are Trying to Solve?

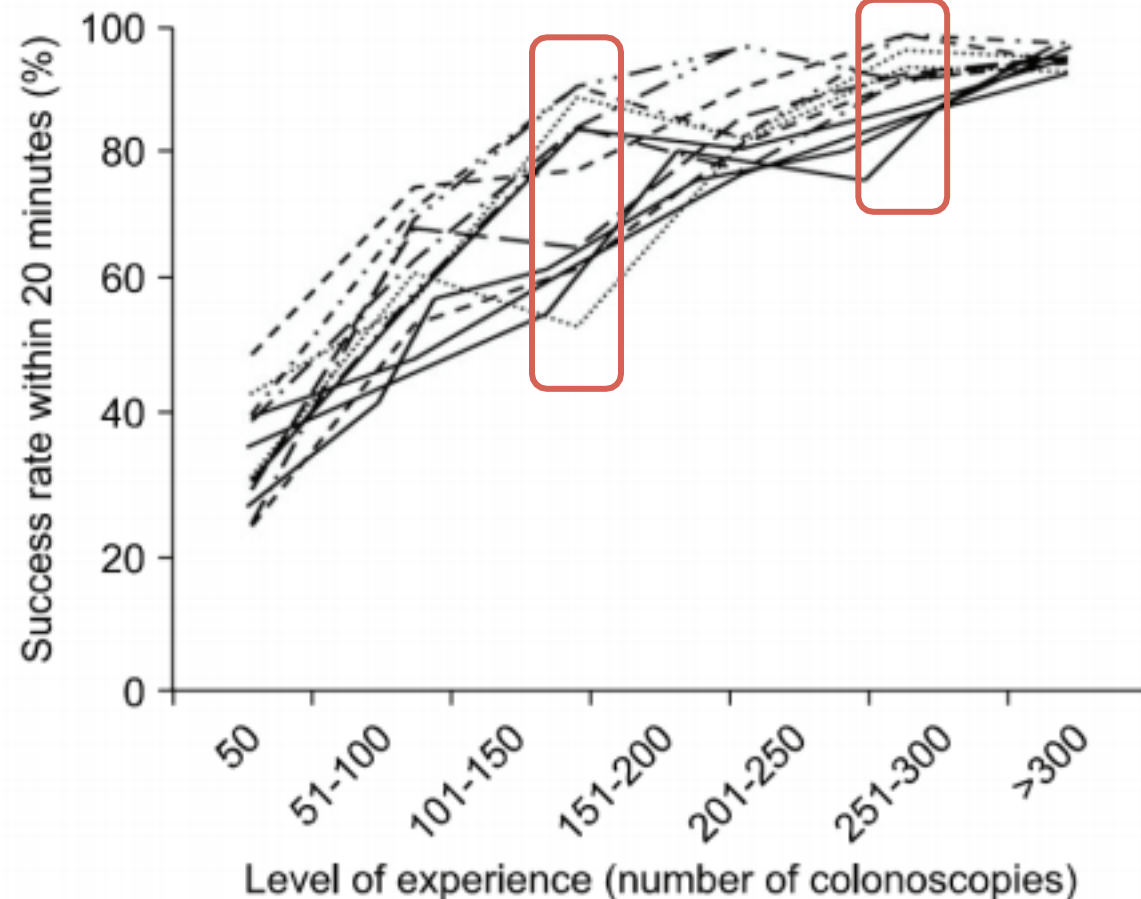
If a resident has completed the required time but is not ready for independence, why should that allow an on-time graduation?

If a resident has demonstrated readiness for independent practice, what is the educational purpose of additional time?

Developmental Curves in Colonoscopy

- A few trainees had the same technical proficiency at scope 150 as they did at scope 250
- Should all these individuals have to train for the same length of time?

Figure 2.1 Number of colonoscopies to attain proficiency



Competency-Based Medical Education (CBME) Defined

CBME is “an approach to *preparing physicians for practice that is fundamentally oriented to graduate outcome abilities and organized around competencies...It de-emphasizes time-based training and promises a greater accountability, flexibility, and learner-centeredness.*”

Promotion in Place

Promotion in Place (PIP) is a specific model of competency-based time-variable (CB-TV) GME where qualified residents who meet explicit competency criteria voluntarily graduate early, become fully credentialed attendings with billing privileges, and practice in a period of “sheltered independence” as an attending in their home institution until what would have been their standard graduation date.

Original pilot funding:

Rationale for PIP-SI in General Surgery

- Have been building competency-based assessment tools (ABSITE, Milestones, EPAs, etc)
 - EPAs implemented for 5 years (2028) at time of proposed PIP-SI pilots
- PIP-SI is the next logical step in exploring CB-TV model in GS:
 - Gently (~6-month change) moves the needle on CB-TV in GS
 - Infuses autonomy (judgement) into surgical training which is both critical and frequently wanting
 - Allows the opportunity to smooth the transition into practice
- Propose to evaluate the outcomes, feasibility, and acceptability of PIP-SI as a model of CB-TV in GS

How To Know Which Trainees Are Ready?



vs.

vs.



Portfolio of performance data

What Are The Current GS Requirements?



**GS potential portfolio
of performance data**

- Trainees must take Qualifying (written) and Certifying (oral) exam
 - QE can be taken in 4th of 5 years of training
- GS EPA Assessments:
 - ✓ 18 EPAs actively being used
 - ✓ Mobile app-based collection
- Milestones
- End of rotation evaluations

General Surgery Has Been Building on CBME Infrastructure

ABS Requirements

Case log minimums met
Certifications: ATLS, FLS, FES
Pass Qualifying Exam

EPA Assessments:
Practice Ready for all EPAs*

**EPAs with fewer than 0.5% of collected assessments will be exempted from this requirement*

ACGME Requirements

Milestone reporting †

End of rotation evaluations:
No concerns about communication, professionalism, or teamwork

Multi-source feedback:
Assess integrity, reliability, knowledge of limitations, ability with flexibility in tool used

† Milestone levels requirements will vary for PIP participation. Additional information available.

PIP-Surgery Timeline

KEY MILESTONES TOWARD IMPLEMENTATION



Adaptation of the PIP Model to General Surgery

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The Blue Ribbon Committee II Report and Recommendations on Surgical Education and Training in the United States: 2024

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VIEWPOINT



National Pilot Will Assess Feasibility of Promotion in Place for Surgical Trainees

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OF SURGEONS

General Surgery CB-TV PIP Model (PIP-General Surgery)



In the Eye of the Beholder: A Stakeholder Analysis of the Value of the “Promotion in Place” Competency-Based Time-Variable Graduate Medical Education Pilot

 Balmer, Dorene F. PhD;  Pusic, Martin V. MD, PhD;  Weinstein, Debra F. MD;  Co, John Patrick T. MD, MPH, MBA;  Goldhamer, Mary Ellen J. MD, MPH

- Qualitative stakeholder analysis program evaluation
- 34 interviews: residents, PDs, service chiefs, chairs, external leaders
- Harvey & Green’s 4 overarching categories of quality in higher ed to conceptualize value
- Promotion in Place:
 - **Fit for purpose:** Supports workforce readiness, sheltered independence, enhances education and trainee satisfaction and well-being
 - **Standards monitoring:** PIP must maintain patient outcomes and resident outcomes
 - **Value for the money:** Cost neutrality important for sustainability, avoid “hidden costs” e.g., assessment
 - **Transformation:** PIP promotes independent decision making, enhances trainee confidence, strengthens assessment practices

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Program Director, reflecting on the Board perspective

“Starts with a personal trust ... they (the Board) had a good amount of personal trust in [the Program Director]”... ‘Well, you're telling us that you're willing to take these early graduates and have them take care of your patients in your department. So, you're vouching for them. I think that means that you really trust your ability to assess them, and so we can trust you on that.’”

Thank you!